

Hospital ER Performance Report

Executive Dashboard Summary & Strategic Clinical Recommendations (2023–2024)

9,216

TOTAL VALID ADMISSIONS

35.3 min

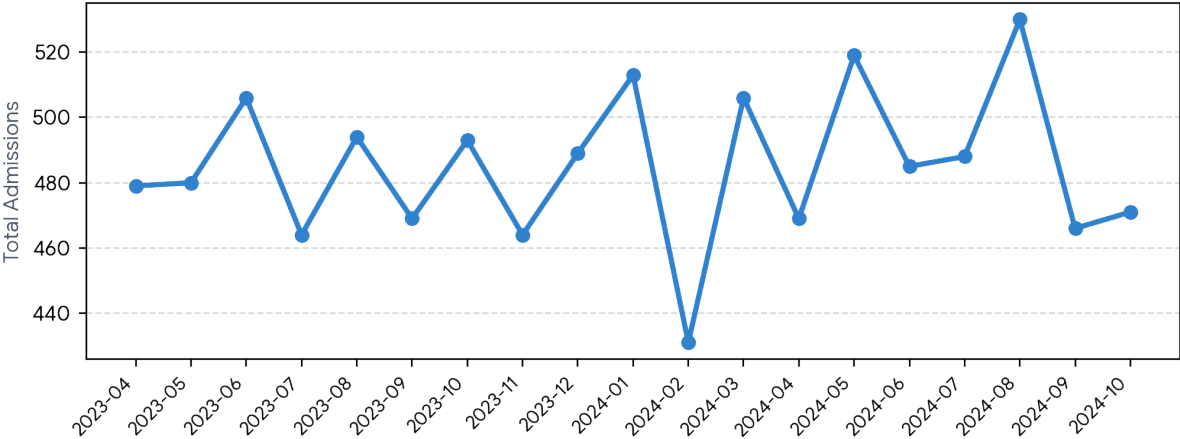
AVG AGGREGATE WAIT
TIME

4.99/10

BASELINE SATISFACTION

OPERATIONAL DEEP DIVE & DATA VISUALIZATION

Monthly Patient Volume Trend (2023-2024)



Traffic Surges & Seasonality

The ER experienced a **12.4% YoY volume surge**. As seen in the trendline, **August** represents the absolute peak operational strain, pointing to highly predictable Q3 seasonality.



The Off-Peak Paradox

In February 2024, despite recording the lowest patient volume, the ER produced the highest average wait times (**36.67 minutes**). This indicates severe internal friction during off-peak scheduling.



Structural Resilience

Despite handling nearly 500 more patients in 2024, the aggregate waiting delta escalated by a mere **26 seconds**, proving the baseline triage infrastructure is remarkably resilient to heavy volume.

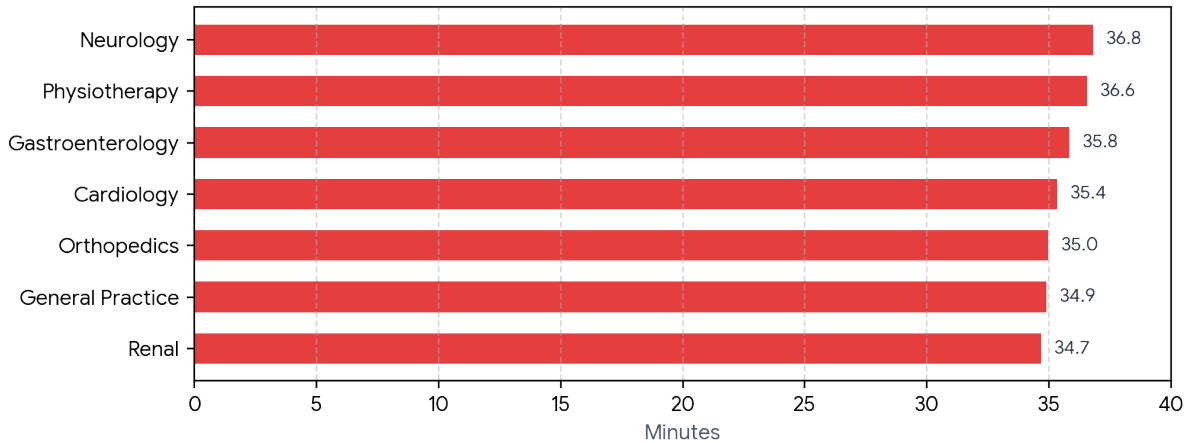


Satisfaction Disconnect

Statistical correlation between Wait Time and Satisfaction is near-zero (-0.02). Patients are scoring the ER based on **clinical care quality** and discharge procedures, not processing speed.

Departmental Bottlenecks & Strategic Actions

Average Wait Time by Department (Minutes)



STRATEGIC BUSINESS RECOMMENDATIONS

Based on the quantitative metrics visualized above, the following data-driven interventions are recommended to Administration:

1. Dynamic Staffing for Peak Q3 Seasonality

Implement an automated, data-driven workforce scheduling index. Scale up on-duty triage officers and nurses by **15% to 20%** specifically during the late July to early September window to proactively neutralize the predictable August volume bottlenecks.

2. Re-engineer Specialized Intake Handoffs

As the chart above proves, **Neurology** and **Physiotherapy** are the primary facility bottlenecks despite low overall volume. Build dedicated fast-track clinical pathways so triage officers can route these patients directly to on-call consultants, bypassing the general queue.

3. Conduct an Immediate Off-Peak Capacity Review

Mandate a review of off-peak scheduling configurations (focusing on the winter/February cycles). Revise shift handover templates to ensure that down-scaled night/winter staffing matrixes do not artificially trap patients in the waiting room when the facility is quiet.

4. Strategic Pivot to Care-Quality over Velocity

Because speed does not correlate with patient satisfaction, reallocate secondary operational budgets away from speed-optimization tech and invest directly into empathy-based patient care training and specialized bedside upgrades, targeting the low-scoring Renal department immediately.